HEALTHCARE AND PUBLIC HEALTH SECTOR DESIGN BASIS FRAMEWORK

Design Basis, Design Basis Accident, and Military-Action Extension Methodology
Function-Primary Facility-Class Architecture — a Hybrid Consumer / Producer Sector

DBA-HPH: Healthcare and Public Health Sector Parent Document

A standalone sector series in the Design Basis for AI Governance in Critical Infrastructure

Version 0.6 — DRAFT

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For review by engineers, regulators, health-system operators, and public-health and policy analysts responsible for the resilience of care delivery, medical supply, and public-health infrastructure.

SUGGESTED CITATION

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This version is a member of CIKR Design Basis Examples, Edition 1 (November 2026) (https://doi.org/10.5281/zenodo.23863986), a dated edition that freezes, for each of fifteen critical infrastructure sectors, a sector framework, a facility class and a reference event as they stood on 1 November 2026. The edition is not falsified by a later revision of this document, whose working master continues to move. It is a working draft, marked DRAFT on its cover.

Section 1 — Purpose, Scope, and Series Architecture

1.1 Purpose

This document establishes the Design Basis Framework for the National Infrastructure Protection Plan (NIPP) Healthcare and Public Health sector. It is the parent instrument for the DBA-HPH series, and it applies the DB → DBA → DBA-MA three-layer analytical progression to the functions on which patient care and population health depend: care delivery, medical materials and supply, and public health and laboratory capability.

1.2 What This Document Does Not Do

This document does not supersede health regulation or accreditation. It does not restate the Bright Line or Command Broker, inherited by reference from FD-BL. For its consumer functions it does not re-scope the physical infrastructure care delivery runs on: the water, electric power, communications, and medical gas that a hospital depends on are governed by the DBA-W, DBA-EN, and DBA-COMM series and inherited here.

1.3 Function-Primary Organizing Principle

Placement follows the consequence chain. The sector is organized by function — the delivery of care, the supply of medical materials, and the maintenance of public-health and laboratory capability — and each function is placed by the patient-harm or population-health consequence its failure produces, sub-indexed by life-safety time-criticality.

1.4 Series Placement — DBA-HPH as a Standalone Sector Series

DBA-HPH is a standalone sector series, a peer of the energy, water, chemical, communications, transportation, financial, and nuclear series. It inherits the DB → DBA → DBA-MA methodology from the DBA-MA parent and the Bright Line and Command-Broker architecture from FD-BL, and it applies the mandatory Military-Action Extension Applicability Determination.

1.5 Healthcare as a Hybrid Consumer / Producer Sector

Healthcare and Public Health is a hybrid sector, and this is its distinctive architectural feature. Its care-delivery function is a consumer function on the model of the financial sector: a hospital produces no physical commodity, and its resilience rests on inherited infrastructure — electric power, potable and dialysis-grade water, communications, and medical gas. Its medical-materials function, by contrast, is a producer function: pharmaceutical and biologics manufacturing, medical-device production, and the cold chain are physical production and logistics facilities with their own design basis, bounded with the Chemical and Transportation sectors. A single sector framework must carry both shapes.

The care-delivery function also stands in a specific relationship to the water and electric series. Hospitals and, most acutely, dialysis centers are already the anchoring life-safety dependency in

the DBA-W framework — DBA-W §8.2 establishes dialysis as the most time-critical dependent facility class — and they carry the Tier-3 designation in the electric series. DBA-HPH is the supply-side dual of that dependency: the facilities for which DBA-W and DBA-EN already elevate are the facilities this framework designs. One further note bears on the eventual reconciliation of the corpus's reference materials: the CIKR sector mindmap does not carry a Healthcare sector, representing it only as a medical-supply-chain fragment; the care-delivery and public-health functions established here are absent there and are a documented gap for that reconciliation.

Section 2 — The Healthcare Sector and Its Regulatory Context

2.1 A Fragmented, Largely-Private, Multi-Regulator Sector

The Healthcare and Public Health sector is predominantly privately owned, highly fragmented, and supervised by a family of federal and state authorities. The Department of Health and Human Services is the Sector Risk Management Agency; the Centers for Medicare and Medicaid Services set the conditions of participation that function as de facto operating standards; the Food and Drug Administration regulates drugs, devices, and biologics; the Centers for Disease Control and Prevention anchor the public-health and laboratory function; and the Administration for Strategic Preparedness and Response leads preparedness and surge. Accreditation through the Joint Commission and state health-department licensure complete the picture, with Health-ISAC providing sector information sharing.

2.2 Regulator-Per-Function Inheritance

Because the applicable authority varies by function, the function-primary taxonomy is the natural structure for the regulatory terrain: care delivery inherits CMS conditions of participation, FDA device regulation, and accreditation; medical materials inherit FDA manufacturing regulation and the hazmat and transport regimes shared with the chemical and transportation sectors; public health and laboratory inherit the CDC and state-health framework; and the coordination function inherits the HHS and ASPR preparedness structure.

2.3 The Regulatory Gap

The sector carries extensive accreditation and emergency-preparedness requirements, but no instrument states a cross-sector engineering design basis in consequence terms — the set of events the care-delivery, medical-supply, and public-health functions must be demonstrated to withstand, expressed as patient-care-continuity and material-supply requirements. Preparedness is measured in plans and drills; a design basis measured in demonstrated withstand capability does not exist.

Section 3 — Function-Primary Taxonomy

3.1 The Function Classes and the Time-Criticality Sub-Index

The sector is decomposed into three physical-or-consumer function classes — care delivery, medical materials and supply, and public health and laboratory — and one coordination class, the sector-coordination layer. Each is sub-indexed by life-safety time-criticality, so that the design basis load a given instance carries scales with how quickly its failure produces patient harm.

3.2 Care Delivery (CD)

The care-delivery function covers hospitals and medical centers, dialysis clusters, and long-term-care facilities. Its governing consequence is loss of patient-care continuity, and its defining feature is that time-criticality varies sharply by dependent population: dialysis cannot be sustained from stored water for more than a few hours, surgical and critical-care functions fail within hours of power or medical-gas loss, and general care degrades over longer intervals. It is a consumer function whose physical substrate is inherited (Section 6.1).

3.3 Medical Materials & Supply (MM)

The medical-materials function covers pharmaceutical and biologics manufacturing, medical-device production, the blood and plasma supply, critical-medication stockpiles, and the temperature-controlled cold chain. Its governing consequence is supply denial of a critical medical material, and it is a producer function: its manufacturing plants are bounded with the Chemical sector (pharmaceutical and biologics production shares the process-release and supply-chain-denial analysis of DBA-CHEM), and its cold chain is bounded with the Transportation sector.

3.4 Public Health & Laboratory (PH)

The public-health-and-laboratory function covers public-health laboratories, diagnostics and disease surveillance, and mass-care and mortuary capability. Its governing consequence is loss of the detection-and-response capacity on which the population-scale management of a health emergency depends — the function whose degradation converts a containable outbreak or mass-casualty event into an uncontained one.

3.5 OpsCo — Sector Coordination (coordination class)

The coordination function is carried by a coordination class spanning the federal preparedness-and-response structure under HHS and ASPR, the Health-ISAC information-sharing mechanism, and the health-information-exchange infrastructure. It is the locus at which the AI-Governance Design Basis is implemented for the sector — the Bright Line placement and Command-Broker obligation for machine-learning and generative-AI systems in clinical decision support, diagnostic imaging, triage, and automated medication and infusion control.

† OpsCo is introduced here as a coordination-class designation; the CD / MM / PH designators were ratified 2026-08-09. The hybrid consumer/producer shape — a care-delivery consumer function beside a medical-materials producer function within one framework — is confirmed here.

3.6 The Patient-Care Continuity Consequence Metric

The energy, communications, and financial series each establish a governing consequence-duration metric. The healthcare analog, proposed here for development, is the patient-care-continuity window: the continuous interval within which a care-delivery function must be sustained before patient harm results, calibrated to the most time-critical dependent population served. It is measured in hours for the dialysis and critical-care cases and its site-specific calibration and Zone-based escalation are to be developed in the care-delivery facility-class instrument. *Care Delivery* (DBA-HPH-CD-FC1) calibrates the window by time-criticality band and by substrate, and adds a second governing measure for the receiving function, surge throughput to definitive care (*Care Delivery* §3.3); this framework takes both up at its next conformance pass.

Section 4 — The Healthcare Sector Design Basis

[Skeleton — to be developed.] The design envelope will be organized by event category with a consequence-severity inclusion criterion consistent with the series.

4.1 Design Basis Principles

The criterion for inclusion is patient-harm or population-health consequence severity — potential for loss of life-sustaining care, denial of a critical medical material, or loss of the public-health capacity to manage a health emergency at population scale — not event likelihood. Candidate bounding events, to be developed: hospital loss of power, dialysis-grade water, or medical gas across the continuity window; cold-chain break and critical-medication supply denial; mass-casualty or pandemic surge beyond capacity; and cyber compromise of hospital operational technology and the electronic health record.

Section 5 — Dual Design Basis: Bright Line and Command Broker

[Skeleton — to be developed on the DBA-W §5.5 model.]

Healthcare is deploying machine learning and generative AI directly into the patient-care path — diagnostic imaging interpretation, clinical decision support, sepsis and deterioration prediction, triage, and automated medication and infusion control. The Bright Line must be placed for any action of a component within scope of FD-LD §4.3 whose incorrect performance could produce patient harm at or above the severity threshold — a medication dose, an infusion rate, a triage disposition, an automated clinical intervention — with the architecturally enforced authorization of a qualified clinician as Command Broker. This is the sector in which the Command-Broker requirement most directly protects individual human life. Determinism decides scope under FD-LD §4.3; it does not decide placement.

Section 6 — Cross-Sector Dependencies and the Consequence Elevation Rule

6.1 Inward View — the Care-Delivery Dependencies

For the care-delivery function the inward view is defining. A hospital depends on the electric series for its life-safety load, on the water series for potable and, critically, dialysis-grade water, on the communications series for its clinical and coordination links, and on medical-gas and generator-fuel supply. Dialysis-grade water is the sharpest of these: it is the dependency DBA-W §8.2 identifies as the most time-critical in the entire water sector, and it is stated here as a design basis condition rather than assumed.

6.2 Producer Boundaries and the Outward Cascade

The medical-materials function is bounded with two producer sectors: pharmaceutical and biologics manufacturing with the Chemical sector, which supplies the process-release and supply-chain-denial analysis, and the cold chain with the Transportation sector. The outward cascade of the sector runs to the population it serves: loss of care delivery or of a critical medical material is a direct life-safety consequence, and the public-health function's degradation amplifies any concurrent emergency across every other sector's workforce and population.

6.3 The Consumer-Function Elevation — Reversed Direction

As in the financial sector, the consumer function reverses the elevation direction: because care delivery runs on inherited infrastructure, a water, electric, or communications asset that is the sole supply path for a hospital or dialysis cluster inherits that facility's life-safety tier. This closes a loop already open in the corpus — DBA-W and DBA-EN already elevate for the hospital and dialysis dependency; DBA-HPH now names those facilities as the sector's own care-delivery class, making the elevation reciprocal and explicit.

6.4 The Boundary with Emergency Services

The pre-hospital response belongs to the Emergency Services series and the receiving facility to this one, and the patient-transfer point is the interface both series state (*Emergency Services Sector Framework* §6.3). On the author's decision of 6 October 2026, medical control is divided on the same line. The clinical authority of the medical director — the protocols and standing orders under which dispatchers and medical crews act, including those approved in advance, and orders given to a crew in the field — belongs to this sector. The dispatch action and the field action taken under that authority, and their design basis, belong to the Emergency Services series (*Public-Safety Answering and Dispatch* §8.3; *Response Forces and Stations* §6.5). The clinical standard that a decision-support system used by a dispatcher or a crew must meet is this sector's to set, and its placement on the Bright Line is made on the consequence of the action it supports, in the series that owns the action. The distribution of patients to hospitals in a mass-casualty surge is a shared interface: the response forces plan for patients who reach hospitals without passing through them and tell the hospitals what is coming, and the receiving hospitals hold the surge capacity and their own distribution of patients (*Response Forces and Stations* §8.3). The

Las Vegas shooting of 1 October 2017, conferred in the Emergency Services series as DBA-ESS-RE-11, shows the interface under load.

On the author's decision of 7 October 2026, the shared interface runs both ways. The response forces tell the hospitals what is coming as far as they know; the hospitals tell command what has arrived, so that the count of patients by hospital, which the response forces require command to hold, is built from the receiving end (*Response Forces and Stations* §4.7, §8.3). The hospitals' side is this sector's. A receiving hospital in a mass-casualty surge counts and reports the patients who reach it, by whatever means they came; is able to receive notification from the response by a means that does not depend on someone answering a radio; and plans for a surge that arrives without notice and without field triage.

The Las Vegas shooting is the evidence, recorded here and not yet conferred in this series. The Nevada Hospital Association's case study records that about eight hundred of the injured found their own way to hospitals, carload after carload arriving without notice; that five hospitals received about 750 patients between them, the nearest, which is not a trauma center, about 150; that several hospitals did not answer the radio when the fire department's dispatch center first tried to notify them; that patients were treated under aliases faster than registration could keep up, linens and airway supplies ran short within hours, and four to six family members arrived for every patient; and that the hospitals' ability to move patients quickly through triage and the emergency department to surgery mattered more than their count of surge beds (*Route 91 Harvest Shooting* §3.4, F-4). On the author's decision of the same date, the event is held as cited evidence and is to be considered for conferral as a reference event of this series when its first receiving-facility class is written.

Section 7 — Military-Action Extension Applicability

[Skeleton.] Medical facilities hold protected status under international humanitarian law, yet hospitals and health infrastructure have been directly affected in recent conflicts, and mass-casualty surge is itself a military-action consequence of attacks on other sectors. The sector-level presumption toward MA applicability is strong for trauma and burn capacity, the medical-materials stockpile, and public-health response capability. Application remains a per-facility determination under the mandatory two-factor test — geolocation and strategic value — with strategic value driven by irreplaceable surge and specialty capacity.

Section 8 — Facility Class Register and Inheritance Model

8.1 Inheritance Model

Facility-class instruments of every type in the DBA-HPH series inherit the threat taxonomy, bounding methodology, consequence-analysis structure, adequacy standards, and Bright Line / Command-Broker architecture established in this framework. The care-delivery class additionally inherits the physical design basis of its substrate from DBA-W, DBA-EN, and DBA-COMM; the medical-materials class inherits the process and transport analysis of DBA-

CHEM and DBA-TRAN. Each instrument adds the health-function-specific consequence chain and recovery architecture.

8.2 Facility Class Register

The following table is the current facility-class register for the DBA-HPH series. DBA-HPH-CD-FC1 has issued as a working draft and the other function classes are at Anticipated status; the inherited physical substrate is shown as governed by the DBA-W / DBA-EN / DBA-COMM series. Designators were ratified 2026-08-09 and are sub-indexed by life-safety time-criticality.

Designator	Function class (function-primary)	Governing characteristic	Status	Primary regulatory anchor
DBA-HPH-CD-FC1	Care delivery — hospitals & medical centers, dialysis clusters, long-term care	patient-care continuity; dialysis most time-critical (hours)	Issued (working draft)	CMS; FDA; Joint Commission; state health
DBA-HPH-MM-FC2	Medical materials & supply — pharma & biologics mfg, medical devices, blood/plasma, stockpiles, cold chain	critical-material supply denial	Anticipated	FDA; boundary with DBA-CHEM & DBA-TRAN
DBA-HPH-PH-FC3	Public health & laboratory — public-health labs, diagnostics, disease surveillance, mass-care	loss of detection & response capacity	Anticipated	CDC; state & local public health
DBA-HPH-OpsCo†	Health-sector coordination (coordination class) — HHS/ASPR, Health-ISAC, health information exchange	preparedness, surge & recovery coordination	Anticipated (architecture)	HHS / ASPR; Health-ISAC
(inherited)	Substrate — water (dialysis-grade), electric power, communications, medical gas	governed by DBA-W / DBA-EN / DBA-COMM	Governed elsewhere	see those series (Care-Delivery inheritance)

Designators were ratified 2026-08-09. Facility-class numbers are sequential across the series; OpsCo is a coordination class and carries no FC number, mirroring DBA-ES-CntlCo. Each function class is sub-indexed by life-safety time-criticality (dialysis / critical-care hours → general care).

Section 9 — Development Status and Open Items

This is the initial working draft of the DBA-HPH sector framework and the second consumer-adjacent framework in the corpus. It establishes the standalone-series placement, the hybrid consumer/producer shape, the function-primary decomposition (care delivery, medical materials, public health and laboratory, and the OpsCo class) with a life-safety time-criticality sub-index, the proposed patient-care-continuity metric, the inheritance of the care-delivery substrate from DBA-W / DBA-EN / DBA-COMM, the producer boundaries with DBA-CHEM and DBA-TRAN, and the reciprocal closure of the DBA-W / DBA-EN hospital-and-dialysis elevation. The design-basis envelope (Section 4), the dual-DB detail (Section 5), and the MA applicability section (Section 7) are stubs to be developed. Open items at this version: develop the patient-care-continuity metric; and flag the Healthcare gap in the CIKR sector mindmap for the eventual reconciliation pass. The boundary with Emergency Services for medical control and mass-casualty surge was stated on 6 October 2026 (Section 6.4); the surge capacity of receiving

hospitals is to be developed with the care-delivery class. At v0.5 Section 6.4 states the hospitals' side of the two-way patient interface and records the Las Vegas hospital surge as cited evidence, its conferral held for the first receiving-facility class. At v0.6, DBA-HPH-CD-FC1 (*Care Delivery*) has issued as the series' first facility-class instrument, written for hospitals as receiving facilities in full with dialysis clusters and long-term care at the depth of the sub-index, and the three reference events it proposed are conferred at Section 9.1. It develops the patient-care continuity window of Section 3.6 and the hospitals' side of the patient interface of Section 6.4. What it notes to this framework is taken up at the framework's next conformance pass: the refinement of the time-criticality sub-index into three bands, the calibration of the continuity window, the event categories it develops ahead of Section 4, the finding that a hospital's identification serves as a targeting aid, its declared residuals, the consequence-basis conditionality check, and its reading that actions taken inside the hospital are placed in the class that owns them while the sector-wide clinical standard stays with OpsCo (*Care Delivery* §10.10). Further open items: decide whether a shared clinical laboratory run for hospitals by a third party belongs to this class, to Public Health and Laboratory or elsewhere (*Care Delivery* §10.6); and write the three reference-event instruments of Section 9.1.

9.1 Conferred Reference Events

On the author's decision of 12 October 2026, this framework confers a reference-event designator on each of the three events its first facility class proposed. A designator commits the series to two things: a row in the corpus-wide DBA Reference Event Register, and a standalone reference-event instrument for that event. Under the determination of 9 October 2026 (*Reference Events Move to Their Own Project*), reference-event instruments are written and held by Project 11, DBA Reference Events; this framework keeps the conferral. None of the three instruments is yet written. The designators are numbered in the order the proposing class lists its events; the number records conferral, not issue. Each designator is sector-primary and single.

From *Care Delivery*, three events are conferred: DBA-HPH-RE-1, the ransomware attack on the Irish Health Service Executive of May 2021; DBA-HPH-RE-2, the loss of power at Memorial Medical Center in New Orleans during Hurricane Katrina, August and September 2005; and DBA-HPH-RE-3, the strike on the Okhmatdyt National Children's Hospital in Kyiv on 8 July 2024. On the author's decision of the same date, Memorial is conferred as an event distinct from the failure of coordination in Katrina, which the Emergency Services series holds as DBA-ESS-RE-8: Memorial is the loss of one hospital's power and its consequence to the patients inside, a different consequence chain in the same storm, and its instrument cites DBA-ESS-RE-8 for the regional context rather than restating it. The Las Vegas hospital surge of October 2017 keeps its single designator in the Emergency Services series, DBA-ESS-RE-11, with healthcare recorded as an impacted sector, and is not conferred here (Section 6.4). The Paris attacks of November 2015 are held as a candidate for the surge chain and are not conferred. The three instruments are owed, and they are Project 11's to write. *Care Delivery* states the three as proposed at its v0.1 and conforms to the conferral at its next revision.

This framework is still a skeleton at Sections 4, 5 and 7. The three instruments read their events back through the design basis of *Care Delivery*, which develops it ahead of the framework, and what they return to the framework is taken up at its next conformance pass.

References

[To be completed.] Governing and inherited instruments: FD-BL and determination annexes; DBA-MA Parent Framework (IAEA L1); DBA Series Gaps and Recommendations (§1.7, MA applicability); DBA-W (dialysis-grade water; §8.2 dialysis dependency); DBA-EN (Tier-3 life-safety load); DBA-COMM (clinical connectivity); DBA-CHEM (pharma/biologics production boundary); DBA-TRAN (cold-chain boundary). Regulatory and coordination: HHS (SRMA) and ASPR; CMS; FDA; CDC; Joint Commission; state health departments; Health-ISAC. Emergency Services boundary: *Response Forces and Stations* (DBA-ESS-RF-FC2) §4.7, §6.5 and §8.3; *The Route 91 Harvest Shooting of October 2017* (DBA-ESS-RE-11), cited by the short title *Route 91 Harvest Shooting*; Nevada Hospital Association, *A Day Like No Other: A Case Study of the Las Vegas Mass Shooting*, 2018. Series instruments: DBA-HPH-CD-FC1 (*Care Delivery*), the series' first facility class; DBA-HPH-RE-1 to DBA-HPH-RE-3, the conferred reference events of Section 9.1, their instruments owed to Project 11. *The Failure of Coordination in Hurricane Katrina* (DBA-ESS-RE-8), cited for the regional context of DBA-HPH-RE-2.

Revision History

0.1 August 2026 — T. Roxey. Initial skeleton. Established DBA-HPH as a standalone sector series with a hybrid consumer/producer shape: a care-delivery consumer function (substrate inherited from DBA-W / DBA-EN / DBA-COMM; the supply-side dual of the DBA-W §8.2 dialysis and DBA-EN Tier-3 hospital anchor) beside a medical-materials producer function (bounded with DBA-CHEM and DBA-TRAN). Adopted the function-primary decomposition (Care Delivery / Medical Materials / Public Health & Laboratory / OpsCo) with a life-safety time-criticality sub-index; proposed the patient-care-continuity consequence metric; stated the reciprocal reversed-direction elevation. Noted that the CIKR sector mindmap carries no Healthcare sector (only a medical-supply-chain fragment) — a documented gap for the mindmap reconciliation. Design-basis envelope, dual-DB detail, and MA applicability sections stubbed for development.

0.2 August 2026 — T. Roxey. Designators ratified; coordination class resolved to OpsCo (HHS/ASPR + Health-ISAC, directive-in-emergency); hybrid CD (consumer) / MM (producer) / PH shape confirmed.

0.3 October 2026 — T. Roxey. Conformed to the October 2026 Edition of the Foundational Definitions on Project 0's work order of 3 October 2026. Section 5 states the scope of Bright Line placement by the determinism carve-out of FD-LD §4.3 rather than as any AI action, and states that determinism decides scope and not placement (P9-11, wording). Functional class is retired at every site under the author's determination of 30 September 2026, each site read for its sense: the decomposition tier reads function class, and the coordination layer reads coordination class (OI-15). No change to the design basis, the taxonomy or the register.

0.4 October 2026 — T. Roxey. Section 6.4 states the boundary with the Emergency Services series on the author's decision of 6 October 2026, issued in the same act as DBA-ESS-RF-FC2 (Response Forces and Stations): clinical authority, including protocols approved in advance, belongs to this sector, and the dispatch and field actions taken under it, with their design basis, to the Emergency Services series; mass-casualty patient distribution is a shared interface. Section 9 records it. Revision-history entries are left as records.

0.5 October 2026 — T. Roxey. Section 6.4 states that the shared interface with Emergency Services at a mass-casualty surge runs both ways, on the author's decision of 7 October 2026: the hospitals count and report the patients who reach them, receive notification by a means that does not depend on a radio being answered, and plan for surge without notice or field triage. The Las Vegas hospital surge is recorded as cited evidence from the Nevada Hospital Association's case study, its conferral held for the first receiving-facility class. Issued in the same pass as DBA-ESS-RF-FC2 v0.2 and from DBA-ESS-RE-11 Finding F-4. Section 9 and the references record it. Revision-history entries are left as records.

0.6 October 2026 — T. Roxey. Section 9.1 is added and confers DBA-HPH-RE-1 to DBA-HPH-RE-3 on the three reference events that DBA-HPH-CD-FC1 (Care Delivery) proposed, on the author's decision of 12 October 2026: the Health Service Executive ransomware attack (May 2021), the loss of power at Memorial Medical Center in Hurricane Katrina (2005) and the strike on the Okhmatdyt children's hospital (July 2024). Memorial is conferred as distinct from DBA-ESS-RE-8, on the author's decision of the same date, and cites it for context; Las Vegas stays with DBA-ESS-RE-11; Paris 2015 is held. Each designator commits the series to a register row and a standalone instrument, none yet written; the instruments are Project 11's. The register at Section 8.2 enters the class as issued. Section 3.6 records the class's calibration of the continuity window and its surge measure. Section 9 records the class's issue and lists what it notes to this framework for the next conformance pass. References add the class, the three designators and DBA-ESS-RE-8. No change to the taxonomy, the designators of the classes, the sub-index or the design basis. Revision-history entries are left as records.